
NATIONAL RESPIRATORY AND ENTERIC VIRUS SURVEILLANCE SYSTEM (NREVSS) DATA USE POLICY AND FORM

A. DATA USE POLICY

The intent of this policy is to assure the availability of NREVSS data to public health researchers while assuring confidentiality for the participating laboratories. The policy will improve the communication and coordination between NREVSS and the data requestor by outlining the major steps related to the release as well as publication and dissemination of the data.

Guidelines for NREVSS data requests:

Requests should be project-specific rather than blanket requests for data (e.g. “respiratory virus circulation from 1995-2005”),

All requests should include a brief description of the **background, objectives, data specifications and publication plan (Refer to Section C)**. Assurances of confidentiality and security of the data are the responsibility of the requesting organization and assurances are to be provided by that organization. For those investigators who may have prior access to the data from another project, work on any new project may not be performed without prior approval.

Before release of the data, the researcher(s) and the Centers for Disease Control and Prevention (hereafter, “CDC”) should discuss and agree upon authorship and the responsibilities of authorship. The primary or senior author should sign this authorship agreement, which specifies authorship, the role of authors, the rules of communication and other essentials.

Before submission of manuscripts, posters or abstracts for publication or other distribution, CDC will receive a **copy for review and comment**. CDC must be given at least three weeks for comment. If a CDC author is included on the publication, additional CDC clearance may be necessary.

B. PROVISIONS FOR USING NREVSS DATA*

- (1) The researcher understands that these data are self-reported by participating diagnostic laboratories. While attempts are made to clean and control the quality of the data, the data provided by the participating laboratories are not independently verified
- (2) The researcher is prohibited from disclosing the identities of participating laboratories without authorization from those laboratories.
- (3) The researcher will use the data only for the requested use. If the researcher wishes to examine unrelated hypotheses using already obtained NREVSS data, another data use request must be submitted to NREVSS.
- (4) The researcher is encouraged to provide NREVSS with a copy of any major publication (e.g. report or journal article) that uses the NREVSS data.
- (5) The researcher will **NOT** release any portion of the data to another party without approval from NREVSS.

Requestor has reviewed the data use provisions and agrees to observe them.



Signature

28 January 2025

Date

*This form expires only one year from the date indicated in section B. If analyses extend beyond a year, the requestor will resubmit this form for NREVSS approval.

C. DATA USE FORM: REQUEST FOR DATA FROM NATIONAL RESPIRATORY AND ENTERIC VIRUS SURVEILLANCE SYSTEM

Section 1: CONTACT INFORMATION

Date: 23 January 2025

Requested by: **Name:** Benjamin John Singer
Name of Institution: University of California, Berkeley
Telephone Number: 4152710981
Email Address: bjsinger@berkeley.edu
Mailing Address: 2121 Berkeley Way, Berkeley, CA 94704

Section 2: DESCRIPTION OF PLANNED RESEARCH

Investigators: Benjamin J Singer, Sara Y Tartof, Joseph A Lewnard

Background: Our research is part of the Center for Forecasting and Outbreak Analytics (CFA) Insight Net program, and is funded under the Innovator grant awarded to Kaiser Permanente Southern California. Our research is part of a project to bring improved modeling and analytics to public health. Infectious disease modelling can be used to improve public health responses in many ways, from forecasting the future impacts of disease to determining the epidemiological characteristics of pathogens. Respiratory infectious diseases have large impacts on health and are considered of particular importance in pandemic preparedness. Models of respiratory diseases have been used during influenza pandemics and the COVID-19 pandemic to improve pandemic response. However, epidemiologists have an incomplete understanding of how many respiratory diseases spread, including the degree and duration of immunity conferred by infection and the origin of age group differences in disease. Developing models based on data gathered in healthcare settings can help to understand these aspects of disease dynamics better. Combining different data sources can help make these models robust and capture more aspects of disease dynamics.

Purpose/Objective: We aim to improve pathogen surveillance and pandemic preparedness by building mechanistic models of respiratory disease spread in Southern California. As part of this work, we will investigate why diseases had different rebound behaviours after relaxation of measures to limit the spread of SARS-CoV-2, and use prevalence data on this different rebounds to better understand how different pathogens spread.

Publication Plans: We will prepare a manuscript describing our conclusions and submit it to a peer-reviewed journal.
(Describe briefly your plans for dissemination)

SECTION 3: LIST OF DATA ITEMS (FIELDS, VARIABLES)

PROVIDE A DETAILED DESCRIPTION OF DATA REQUESTED. INCLUDE GEOGRAPHIC AREA, GEOGRAPHIC UNIT OF ANALYSIS (REGION, DIVISION, STATE), AND TIME PERIOD. FOR ANY OTHER CRITERIA, PLEASE LIST THE SPECIFIC VARIABLES.

REQUEST 1

Geographic Unit of Analysis (select one)		Census Division and HHS Region	
Please specify which, if not all (i.e. <i>Census Region: South</i>) All regions.			
Time Period: (mm/dd/yyyy)		Begin Date: 01-01-198	End Date: 10-01-202
Time Unit: (Select)		Weeks	
Virus (Select all that apply)			
Respiratory Syncytial Virus		Antigen, Culture and PCR	
Influenza	FluA and FluB	Antigen, Culture and PCR	
Human Parainfluenza	PIV 3	Antigen, Culture and PCR	
Rotavirus			
Enteric Adenovirus			
Respiratory Adenovirus		Antigen, Culture and PCR	
Human Metapneumovirus		Antigen, Culture and PCR	
Human Coronavirus			
Rhinovirus/Enterovirus			
List Additional Data Criteria:			
Delivery Instructions			
Data Format:		CSV	
Method:		Electronic Mail	
Recipient Name:		Benjamin John Singer	
Recipient Address (electronic address):		bjsinger@gmail.com	
Weekday:			
Time (h: mm tt):			

REQUEST 2

Geographic Unit of Analysis (select one)				
Please specify which, if not all (<i>i.e. Census Region: South</i>)				
Time Period: (mm/dd/yyyy)		Begin Date:		End Date:
Time Unit: (Select)				
Virus (Select all that apply)				
Respiratory Syncytial Virus				
Influenza				
Human Parainfluenza				
Rotavirus				
Enteric Adenovirus				
Respiratory Adenovirus				
Human Metapneumovirus				
Human Coronavirus				
Rhinovirus/Enterovirus				
List Additional Data Criteria:				
Delivery Instructions				
Data Format:				
Method:				
Recipient Name:				
Recipient Address (electronic address):				
Weekday:				
Time:				

PLEASE ACKNOWLEDGE EACH BELOW by checking the appropriate box below		
We agree to adhere to the policies and procedures set forth in Part A and Part B above	Yes	☐
We acknowledge that this data cannot be used outside of the scope presented within this document	Yes	☐
We agree to acknowledge the "Centers for Disease Control and Prevention, National Respiratory and Enteric Virus Surveillance System" in all manuscripts, presentations or abstracts that use the requested data.	Yes	☐
Any manuscripts, presentation or abstracts will be sent to the CDC for review prior to publication.	Yes	☐

This data use form only applied to those uses of the requested NREVS data described above. The uses of this data are permitted for one year from the date listed in Section 5. Additional analyses of the data requested in this form must be described in a new data use agreement and approved by NREVSS.

Print Name Benjamin John Singer

Title Dr

Organization University of California, Berkeley

Signature 

Date 28 January 2025

FOR CDC INTERNAL USE
Description of data release

Email form to nrevss@cdc.gov